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OncoHealth Platform Interoperability - Key Work to Be Done

CMS-0057-F compliance

Overview

Beginning in January 2026, OncoHealth and its payer customers will need to comply with certain aspects of CMS-0057-F, a final rule which requires payers to make available data such as prior authorization (PA) information (including case status) and provider and patient data via APIs. Given that OncoHealth is not a payer, most of the net new work for these requirements will be related to getting the necessary data to OH's customers in a timely, standardized way so they can serve up the necessary data via their own APIs. These payers will be responsible for hosting the Patient/Provider Access and bi-directional Prior Authorization APIs that are required under this final rule and are also responsible for API educational resources and payer-to-payer APIs.

Work to be done

The key work items for OncoHealth to comply with CMS-0057-F are likely to include:

- Add the concept of "pending" interim statusing to OneUM PA workflow
- Build API to pass prior authorization status updates to payers immediately after each case status update and decision (not in end-of-day batches as is currently common), including comprehensive auth details such as units, codes, and requested duration
    - This should follow the FHIR Da Vinci Prior Authorization Support (PAS) Implementation Guide, which has been recommended by CMS as the preferred method for compliance with 0057
- Build a configuration flag for case IDs used in outbound PA status updates to accommodate payers who cannot accept the same case ID twice (e.g. send "1234-pending," then "1234")
- Build mechanism to receive inbound auths from payers in standardized format (e.g. via X12 278 or other format, or potentially via manual intake initially), along with landing place in OneUM (e.g. inbound queue)
    - This should also follow the FHIR Da Vinca PAS IG referenced above
- Update contract/SOW language to define outputs/formats and responsibilities, and coordinate with each payer to confirm ingestion capabilities and any plan-specific constraints

Scoping and level of effort

The above work is expected to be a medium to large project, with a very rough estimate of 1-2 quarters of work for at least one dedicated engineering team and fractions of several others; this timeline may be longer depending on the level of effort required to discover and accommodate customer-specific requirements. In order to produce a more accurate estimate and implementation plan, the following design and discovery must be done:

- Conduct inventory of current-state OncoHealth interfaces, file feeds, batch cadences, auth ID behaviors, exception cases (plans that can't accept updates), and intake capabilities to determine what functionality gaps exist that would impact compliance (including gaps beyond those known today)
- Decide on API/data transmission specs (e.g. FHIR, X12 278, etc.), auth model (OAuth/SMART), and event timing expectations
- Determine the exact fields that must be part of case updates, including pending and final decisions, appeals, drug-level status changes, code systems involved, and any applicable reason codes and identifiers
- Evaluate project scope per payer customer, including which APIs/endpoints will be used, which lines of business are affected, expected volumes, and payer- or plan-specific requirements that deviate from the expected norm